

WEIGHT LOSS

Compounded GLP-1 Sourcing & Documentation

Pharmacy vetting, disclosure, lot tracking and the paper trail regulators ask for first

DOCUMENT WL-006	VERSION 1.0	EFFECTIVE January 2026	REVIEW CYCLE Annual
APPLIES TO Medical director, practice manager, prescribing clinicians	OWNER Medical Director	CLASSIFICATION Confidential	SUPERSEDES All prior versions

1 Purpose & Scope

This protocol governs the sourcing, storage, disclosure and traceability of compounded GLP-1 preparations. It is the most scrutinized area of weight-loss practice and the document most likely to be requested by a board, a carrier or an attorney.

LEGAL REVIEW REQUIRED

Whether compounded GLP-1 preparations may lawfully be prescribed or dispensed at any given time depends on federal drug shortage status, FDA enforcement posture, state pharmacy law, and ongoing litigation — all of which change faster than any template. **This protocol does not tell you whether to use compounded product.** It tells you what to document if your medical director, advised by a healthcare attorney, decides that you may.

RECORD THE BASIS, AND REVIEW IT

Whatever the current status, record the specific basis on which compounded product is being used, the date that determination was made, who made it, and the date it will be reviewed. An undated assumption from two years ago is not a basis.

2 Pharmacy Vetting

Category	What it means	Practice implication
503A compounding pharmacy	Compounds for an identified individual patient against a valid prescription.	Patient-specific prescriptions required. Office stock is generally not permitted.
503B outsourcing facility	Registered with FDA, subject to CGMP requirements and FDA inspection. May produce batches without patient-specific prescriptions.	Permits office stock where state law allows. Higher manufacturing standard.

Vetting file — obtain before the first order and refresh annually

- ☐ Current pharmacy or outsourcing facility license for its home state, verified at primary source.
- ☐ Non-resident pharmacy license or registration for your state, where required.
- ☐ FDA registration confirmed for any 503B outsourcing facility.
- ☐ Most recent inspection outcome, and any FDA warning letters or state disciplinary action — checked, not assumed.

- ☐ Sterility and endotoxin testing practices for sterile injectables, in writing.
- ☐ Potency and identity testing, with certificates of analysis available on request.
- ☐ Active pharmaceutical ingredient source and country of origin.
- ☐ Beyond-use dating methodology and its basis.
- ☐ Recall procedure and notification pathway to your practice.
- ☐ Insurance and indemnity position.
- ☐ Named contact for quality queries and adverse event reporting.

REFUSE OUTRIGHT

Product marketed as 'research chemical' or 'not for human use'. Product sourced from an unlicensed reseller, a peptide website, or a social media supplier. Product where the pharmacy will not supply licensure or testing documentation. Semaglutide salt forms such as semaglutide sodium or semaglutide acetate, which are not the same active ingredient as the approved base and have been the subject of specific FDA concern. No commercial pressure justifies any of these.

3 Receiving, Storage & Traceability

- 1 **Inspect on receipt.** Packaging intact, cold chain indicators within range, labeling complete and legible, beyond-use date acceptable for expected use.
- 2 **Reject and quarantine** anything with a compromised seal, a temperature excursion, missing labeling, or visible particulates or discoloration. Do not use pending a decision by the medical director.
- 3 **Log receipt** in the product register: pharmacy, product, concentration, lot, quantity, beyond-use date, receipt date, and receiving staff member.
- 4 **Store per labeling.** Refrigerated, protected from light, in a monitored unit with a daily temperature log.
- 5 **Record every dispense or administration** against the lot: patient, date, dose, and administering or dispensing clinician.
- 6 **Reconcile the register monthly** — quantity received against quantity used, wasted and remaining. Investigate any discrepancy.
- 7 **Log waste and expiry disposal** with a witness signature.

TRACEABILITY IS THE WHOLE POINT

If a compounding pharmacy issues a recall, you must be able to identify every patient who received product from the affected lot, and contact them, within a day. If your records cannot do that, the sourcing program is not adequate no matter how good the pharmacy is.

4 Patient Disclosure & Consent

Patients frequently do not know they are receiving a compounded preparation rather than branded product, and many assume the two are equivalent. Disclosure must be explicit, written, and made before the first dose.

Required disclosure elements

- That the preparation is compounded and is not an FDA-approved product.
- That compounded preparations are not reviewed by FDA for safety, efficacy or manufacturing quality in the way approved products are.
- The name of the compounding pharmacy or outsourcing facility.
- The concentration supplied, and that it may differ from branded presentations.
- That the branded FDA-approved alternative exists, and its comparative cost.
- Known risks, including that potency and content may vary between compounders.
- Reported dosing errors arising from unit-versus-milligram confusion, and the specific instructions the patient must follow.
- How to report a suspected problem, and to whom.

DOSING ERROR PREVENTION

Overdoses have been reported where patients or staff confused milligrams with insulin syringe unit markings, or where a new vial had a different concentration from the last. Every dispensed vial must be labeled with the concentration and the dose in both milligrams and the corresponding volume. Confirm the concentration every time a new lot is opened. Never let a patient calculate their own conversion.

5 Adverse Events & Recall Response

- 1 **Quarantine immediately.** Remove the affected lot from service and physically separate it.
- 2 **Notify the medical director** the same day.
- 3 **Identify every patient** who received product from the affected lot using the product register.
- 4 **Contact affected patients** with clear instructions on whether to stop, and what symptoms to watch for.
- 5 **Notify the compounding pharmacy** and request their investigation and response in writing.
- 6 **Report to FDA MedWatch** where an adverse event or quality defect is involved.
- 7 **Retain the product** and its packaging rather than discarding it — it may be needed for investigation.
- 8 **Document everything**, including the decision-making and the timeline, and review whether the pharmacy remains suitable.

6 Records to Retain

- ☐ Pharmacy vetting file, current and superseded versions, with annual refresh dates.
- ☐ Documented basis for compounded use, with its determination date and review date.
- ☐ Certificates of analysis where obtained.
- ☐ Product register: receipt, lot, beyond-use date, dispensing and reconciliation.
- ☐ Refrigerator temperature logs.

- ☐ Signed patient disclosure and consent for every patient receiving compounded product.
- ☐ Waste and expiry disposal logs with witness signatures.
- ☐ Adverse event and recall records with the full response timeline.

7 Quick Reference

QUICK REFERENCE CARD

Compounded product — non-negotiables

1. **Vetting file complete** before the first order; refreshed annually.
2. **Basis for compounded use recorded and dated.** Review on schedule.
3. **Refuse** research chemicals, unlicensed resellers, salt forms.
4. **Log every lot in and every dose out.** Recall readiness in one day.
5. **Written disclosure signed** before the first dose. Every patient.
6. **Label mg and volume.** Never let anyone convert from syringe units.
7. **Confirm concentration** whenever a new lot is opened.
8. **Recall: quarantine, identify, contact, report, retain the product.**

Print, laminate and post at the point of care

8 Medical Director Authorization

This protocol is adopted as a standing order for the practice named below. By signing, the medical director confirms the protocol has been reviewed against current clinical guidance, applicable state scope-of-practice rules, and the training level of the staff authorized to perform it.

PRACTICE NAME	LOCATION / SITE
MEDICAL DIRECTOR — PRINTED NAME & CREDENTIALS	LICENSE NUMBER & STATE
SIGNATURE	DATE ADOPTED / NEXT REVIEW DATE

Template notice. This document is an editable template provided for educational and operational guidance. It is not legal advice, does not constitute a physician–patient relationship, and does not guarantee regulatory compliance or the outcome of any inspection. Drug doses, concentrations and device settings must be verified against current manufacturer labeling and prevailing clinical guidance before use. A licensed medical director must review, customize and sign this protocol before it is placed into

service, and must confirm that every task assigned within it falls inside the scope of practice of the personnel performing it in your state.

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